Obstetric transfer to theatre for COVID-19 suspected/positive patients v5

Minimum staff will transfer patient. Staff to wear appropriate PPE prior to entering room and throughout the transfer. Patient will wear facemask or remain on Oxygen mask continuously till back in the room. Clean patient's hands and Transfer onto trolley with hands. A clean sheet to cover patient and her hands.

An asymptomatic partner will wash hands and can come to theatre with PPE and surgical mask.

The doors of the auxiliary theatre and obstetric theatre will be opened by theatre staff and the patient will pass through the cleared path without touching anything. The patient will transfer on to the operating table. The trolley will be moved back through the auxiliary theatre. The trolley and auxiliary theatre will then be cleaned with Actichlor™ & Clinell™ wipes.

Anaesthetist will be very confident that the spinal/epidural will be adequate and if not will start with a GA.

Surgeons will ensure that the operation will take the minimum amount of time to complete safely. i.e. not appropriate to be teaching because if the spinal wears off will need to convert to a GA which will need all to be in FFP3

Minimum number of staff in theatre: Obstetric surgeon and assistant, Scrub staff, Clean and Dirty Runners, Anaesthetist and ODP, Midwife

If GA everyone in theatre to be in FFP3 PPE

If Spinal Epidural everyone in PPE with Surgical Face Mask

If baby well give to mother or stay in cot

If baby needs resuscitation it will go to Resuscitaire™ in Recovery room.

At the end of the operation – all who are scrubbed to help transfer the patient from the table to the trolley.

If GA patient will be extubated and recovered in theatre and not transferred until fully awake.

When stable, patient is transferred out via auxiliary theatre. Main obstetric and auxiliary theatres are then cleaned with Actichlor™ and Clinell™.